

LABORATORY RESULTS

Patient: Harvey Specter (PAT147)

Test Date: March 5, 2026

Ordering Physician: Dr. James Martinez, MD

HORMONE PANEL (Due to Fatigue & Stress Complaints)

Testosterone

- Total Testosterone:** 348 ng/dL (Low) [Normal: 300-1,000]
- Free Testosterone:** 6.8 pg/mL (Low) [Normal: 9.0-30.0]
- Sex Hormone Binding Globulin (SHBG):** 52 nmol/L (Normal)

Cortisol

- Morning Cortisol (8 AM):** 22.5 µg/dL (Elevated) [Normal: 6-23]
- Evening Cortisol (4 PM):** 12.8 µg/dL (Elevated) [Normal: 3-16]
- DHEA-S:** 185 µg/dL (Low-Normal) [Age-adjusted: 200-500]

Thyroid (Follow-up)

- TSH:** 3.8 mIU/L (High-Normal)
- Free T4:** 1.0 ng/dL (Low-Normal)
- Free T3:** 2.4 pg/mL (Low-Normal)
- Reverse T3:** 28 ng/dL (Elevated) [Normal: 9-27]

Vitamin D

- 25-Hydroxyvitamin D:** 28 ng/mL (Insufficient) [Optimal: 30-100]

COMPLETE BLOOD COUNT

- WBC:** 7.8 K/uL (Normal)
- RBC:** 5.2 M/uL (Normal)
- Hemoglobin:** 15.5 g/dL (Normal)
- Hematocrit:** 46% (Normal)

PSA (Prostate Screening)

- PSA:** 0.8 ng/mL (Normal) [Normal: <4.0]

INTERPRETATION

Low testosterone levels with elevated cortisol suggest chronic stress and possible HPA axis dysfunction. Suboptimal thyroid conversion (elevated reverse T3) may be contributing to fatigue. Vitamin D insufficiency noted. These findings consistent with high-stress lifestyle and inadequate recovery. Recommend testosterone replacement consideration, stress management interventions, and thyroid support.

Reviewed by: Dr. James Martinez, MD

Endocrinology

Date: March 5, 2026

These laboratory results should be interpreted in the context of the patient's clinical history and under the guidance of a licensed healthcare professional.